BASELINE → TREATMENT → FOLLOW-UP

My treatment, tests and checks

Source-required blood tests are distinguished from tests ordered for a particular symptom or disease plan. Imaging is not interchangeable with blood safety tests.

Check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests	Blood count, kidney and liver function, glucose and thyroid tests; hepatitis B surface antigen/core antibody and hepatitis C; tumour PD-L1 CPS and dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) assessment before fluorouracil. Review previous immune problems, transplants, medicines and infection. Audiology and measured creatinine clearance when indicated; assess hydration, hearing and nerves.	Blood count, kidney/liver tests and glucose each cycle; thyroid tests every 3–6 weeks and sooner if symptoms require. Ask about bowel, breathing, skin, hormone and other immune symptoms before every dose. A normal blood test does not exclude immune toxicity.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Continue reporting possible immune effects after the last dose. Follow unresolved blood, kidney, hearing, nerve or endocrine problems and any prescribed steroid taper or hormone replacement. The team sets disease assessment scans and follow-up; this protocol does not mandate a universal imaging interval or routine blood tumour-marker schedule.
Treatment and appointments	Confirm the exact diagnosis, intent, prescription and calendar.	Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time.	Use the prescribed support plan and contact route.	Pembrolizumab 200 mg over 30 minutes every 21 days continues after the combination phase when appropriate, until progression or unacceptable toxicity under this regimen. A six-cycle chemotherapy record is not a six-cycle total course; the protocol does not impose an automatic 24-month stop.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing	Agree oral care and assess weight, intake and swallowing. If radiation is planned separately, arrange its dental preparation with the team.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.
Imaging and tumour assessment	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00706 v3: official regimen

Tests in your plan

Full blood count

WHEN

Before every cycle

Checks cells that fight infection, carry oxygen and help stop bleeding.

Kidney and liver tests

WHEN

Before every cycle

Support safe dosing and detect organ problems.

Thyroid tests

WHEN

Every 3–6 weeks during pembrolizumab; sooner if indicated.

Can detect immune-related hormone changes before or with symptoms.

THE WHOLE TREATMENT JOURNEY

Pembrolizumab + cisplatin + fluorouracil: treatment and checks

First-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma in adults with tumour PD-L1 CPS ≥1.

Every 21 days: pembrolizumab and cisplatin on Day 1; fluorouracil over Days 1–4. Up to 6 combination cycles, then pembrolizumab alone while appropriate. Every dose depends on safety and benefit review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Blood and organ tests NCCP	Blood count, kidney and liver function, glucose and thyroid tests; hepatitis B surface antigen/ core antibody and hepatitis C; tumour PD-L1 CPS and DPD assessment before fluorouracil. Review previous immune problems, transplants, medicines and infection. Audiology and measured creatinine clearance when indicated; assess hydration, hearing and nerves.	Blood count, kidney/liver tests and glucose each cycle; thyroid tests every 3–6 weeks and sooner if symptoms require. Ask about bowel, breathing, skin, hormone and other immune symptoms before every dose. A normal blood test does not exclude immune toxicity.	Contact the team if symptoms develop; extra tests may be needed before the next visit.	Recheck relevant results after a hold or changed plan; previous toxicity remains relevant.	Continue reporting possible immune effects after the last dose. Follow unresolved blood, kidney, hearing, nerve or endocrine problems and any prescribed steroid taper or hormone replacement. The team sets disease assessment scans and follow-up; this protocol does not mandate a universal imaging interval or routine blood tumour-marker schedule.
Treatment and appointments NCCP	Confirm the exact diagnosis, intent, prescription and calendar.	Pembrolizumab 200 mg is infused over 30 minutes, followed by cisplatin 100 mg/m² over 2 hours with prescribed hydration on Day 1. Fluorouracil is prescribed as 1,000 mg/m² each day by 22-hour infusion on Days 1–4, or 2,000 mg/m² over 48 hours twice. Staff confirm pump changes and disconnection from the actual connection time.	Use the prescribed support plan and contact route.	Pembrolizumab 200 mg over 30 minutes every 21 days continues after the combination phase when appropriate, until progression or unacceptable toxicity under this regimen. A six-cycle chemotherapy record is not a six-cycle total course; the protocol does not impose an automatic 24-month stop.	Record the final dose and follow-up appointments.
Eating, mouth and swallowing NCCP	Agree oral care and assess weight, intake and swallowing. If radiation is planned separately, arrange its dental preparation with the team.	Review pain, weight, hydration and safe swallowing.	Report poor intake, choking, weight loss or inability to drink promptly.	Dietitian and speech/language plans adapt to symptoms and recovery.	Continue dental, nutritional and swallowing follow-up as needed.

Care / check	Before starting	Treatment visit	Between visits	Phase / dose review	After treatment
Imaging and tumour assessment NCCP	Confirm staging and any relevant tumour tests; these are distinct from blood safety tests.	Discuss symptoms and treatment response.	The team arranges scans or examinations when indicated.	Use the agreed assessment to guide the next phase.	Record scan and clinic dates; this regimen does not impose a single timetable for everyone.

What each check is for

1 Combination treatment

Up to 6 cycles · every 3 weeks

Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

2 Pembrolizumab alone

Every 3 weeks · ongoing review

Pembrolizumab and chemotherapy aim to control head and neck cancer that has spread or returned and cannot be removed. Your team explains the expected benefit, uncertainty and alternatives for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-07 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.45